



TRANSITIONS

It's not uncommon for some of our consumers to have trouble transitioning from one setting to another, from one task to another, and/or from one staff to another. Conduct issues ranging from anxiety to refusals to agitation to aggression to AWOL can result from this issue. In extreme forms people have refused to move for hours and some have shown extreme fight/flight reactions and have become a danger to themselves and others. One result of the failure to transition is that people may be unable to access services, support and experiences that they would enjoy and which could greatly benefit them.

Arroyo has been referred people where their transition issues are the principal reason for a day program with behavioral support. Once this issue is successfully addressed, the person often showed remarkable progress and enjoyed our program.

Who are the people with transition issues?

People too often are not considered when plans are made, when changes in those plans occur, and when routines are broken. Staff may not realize that the program participant has had bad previous experiences in certain settings. Staff may not adjust plans when program participants don't feel well. Too often people are just rushed, bullied, uninformed and denied the right to say "no!", "not right this minute", or even "what's in it for me!" Another big issue is that there is no plan or tolerance for when people fail to transition at first.

Some people are predisposed to transition issues. They include folks with the following diagnoses and/or conditions:

DIAGNOSIS/CONDITION	TRANSITION ISSUE
Autistic Spectrum Disorder	Strong preference for routine and difficulty handling novel and changing circumstances. They may also be sensitive to some environmental features (lights, sound, waiting in line, crowds, even smell)

Down syndrome	Strong preference for familiar people and routines and difficulty handling novel and changing circumstances. They can refuse to move for hours (including dropping weight)
Obsessive/Compulsive disorder	They may not transition until their routines are completed, checked and possibly repeated
Thought Disorders (particularly with paranoid issues)	A sense of not being safe, of not trusting others, of being overwhelmed by changes
Mood Disorders (particularly depressive)	They may withdraw, refuse to participate, refuse to move
Epilepsy (after big seizures)	They may be exhausted, disoriented and even agitated following big seizures
Heavily Sedated People	They may fall asleep, stagger when walking and have no interest in programming

CAUSES AND TREATMENTS FOR DIFFICULTY WITH TRANSITIONS:

General Causes:

CAUSE	TREATMENT
Previous bad experiences in similar circumstances	Learn history to determine what worked and failed in the past. Avoid recreating past failures by being sensitive to preference and signs of intolerance
Rigid expectations for sameness	Try to keep many things the same while changing only a few things at a time. Allow for observation of others first, permit partial participation, provide escape options, quit while people are still successful, and provide heavy incentives. Plan for an option should there be a failure to transition
Intolerance for the unfamiliar	Make them familiar first with pictures, stories, allow them to see first, lower demands and expectations, leave early, allow refusal
Sensitivity to crowds, lights, noise	Avoid the worst of it; provide adaptations (head phones, sun glasses, etc.).
A successful strategy for taking control, getting attention, and/or extracting concessions	Know history, avoid power struggle, give up control, provide incentives, withhold big attention, wait it out, stimulus change, (have someone else try to move them) recognize and plan for failure to transition.

Fears of being harmed, lack of trust, acute anxiety	Build trust, address the reality of the fears, and teach relaxation and desensitization
A strong dislike of a program that moves from place to place and uses community settings	Limit the number and type of transitions in their program and build tolerance over time

Morning Transitions:

CAUSE	TREATMENT
Fatigue, illness	Work with home staff to promote enough rest, adjust pickup time, and allow for partial participation. Insist on nonparticipation at times (consult Director)
Being rushed, treated rudely, or confrontationally	Try to calm things down, lower expectations; provide incentives, report abusive staff, build trust and friendships
Poor relationships with home staff and/or family.	See above. Try to provide support to staff by working with them (not for them).
Lack of support for attending the program.	Build support for the program or report issues to the Arroyo Director and inform Regional Center caseworker.
Competing contingencies.	Try to make staying home less attractive
Avoiding getting ready in the AM (bathing, grooming, eating).	Schedule bathing at night, finish grooming in DP, provide cheap breakfast.
Stress in AM.	Eliminate it as much as possible

Setting to Setting Transitions:

CAUSE	TREATMENT
They don't want to leave something they like.	Plan ahead in AM, communicate plans, provide incentives, be patient, avoid settings where always happens, try stimulus change (someone else inviting them to leave).
They don't know where they are going next.	See above, provide enhanced schedule (words, pictures, symbols). Have them involved in planning and revisions.
They don't like where they are going next.	Make what follows the worst activity the best activity.

They are sensitive to a break in routine.	Apologize, slow down transition, allow for partial participation, and heavily reinforce any tolerance.
They refuse to enter the car.	Try to calm things down by slowing the expectation for transition, build rapport first, and provide incentives (they can sit where they want, choose the radio station, etc.). Keep things very calm and encouraging when they try to move toward the car. Use child safety locks for safety in case they try to exit a moving car and don't travel far. Never place an agitated person in your car as they may damage it and Arroyo will not pay for the damage. Call the office if they fail to transition to get support.
They refuse to exit the car:	Place the car out of the sun, open the windows and have everyone exit the car who is willing (take your keys). Move the group a short distance away from the person and wait. Do not look at or discuss the situation with the person in the car and give them a lot of attention when they do leave.

Staff to Staff transitions:

CAUSE	TREATMENT
They are very attached to their former staff person.	Respect this and avoid needless changes, have them meet others while with favorite staff who also treat them well, explain the need for change and be calm and patient.
They never met the new staff person.	The new staff must be extra nice, use reinforcement more and better, try to follow old routines to limit other changes, and give the relationship time to build.
They were not prepared in advance for the change.	See above
They know and dislike the new staff and/or their consumers.	Respect opinions, seek resolution of grievances